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Bwrdd Iechyd Prifysgol
Betsi Cadwaladr
University Health Board

ANAESTHESIA, CRITICAL CARE & PAIN MANAGEMENT DIRECTORATE

OBSTETRIC ANAESTHESIA WRITTEN CONTROL DOCUMENT

SPINAL AND EPIDURAL ANAESTHESIA IN PARTURIENTS ON ANTICOAGULANTS

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Contents

1. Introduction and purpose ……3
2. Scope……..……3
3. Points to consider……...3
4. References……..3

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1. Introduction and Purpose

Pregnancy results in an increased tendency to develop a deep vein thrombosis due to an increase in the concentration of clotting factors. As a result thromboprophylaxis measures need to be taken whenever a parturient becomes immobile.

2. Scope

This guideline is intended as a reference for anaesthetic trainees and midwifes.

3. Points to consider

3.1 Enoxaparin (Clexane) is the preferred anticoagulant of choice in a dose of 40mg subcutaneously once daily (administer 20mg if total body weight < 50Kg & 0.6 mg/Kg total body weight if > 90Kg) for thromboprophylaxis or a 1mg/Kg dose twice daily for full anticoagulation.

3.2 The following rules should be followed when considering performing spinal or epidural anaesthesia in a patient taking enoxaparin :

- Only perform a spinal or epidural if 12 hours or more has elapsed since giving the last thromboprophylaxis dose of enoxaparin
- Instruct the midwife not to give the next dose of enoxaparin until at least 4 hours has elapsed since taking an epidural catheter out or performing a spinal block
- Patients on full anticoagulation doses of enoxaparin (1mg / Kg bd.) should not receive a spinal or epidural until at least 24 hours has elapsed since the last dose of enoxaparin or until a normal serum anti factor Xa assay is achieved. These patients should be offered a fentanyl PCA for labour analgesia if they are deemed unsuitable for an epidural based on the above criteria
- Patients who are given unfractionated heparin for thromoprophylaxis at a dose of 5000 iu subcutaneously twice or three times daily (e.g. due to allergy to enoxaparin) or are on an intravenous heparin infusion for full anticoagulation are safe to have an epidural or spinal anaesthetic if they either have a normal APTT ratio, or more than 4 hours has elapsed from being given their last dose of heparin or discontinuation of a heparin infusion.

4. References

1. Association of Anaesthetists of Great Britain and Ireland and Obstetric Anaesthetists Association and Regional Anaesthesia UK. *Regional Anaesthesia and Patients with Abnormailities of Coagulation (2013)* AAGBI, London

2. RCOG Green-Top Guideline No. 37a.Reducing the Risk of Venous Thromboembolism during Pregnancy and the Puerperium (2015). RCOG, London